



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : صفيه صبحي مصطفى محمد

العنوان : عبد المنعم

الحي / المدينة : المطرية

المحافظة : القاهرة

تاريخ تسجيل الإستمارة : 10-10-2026

رقم البطاقة : 27612132101822 رقم التليفون : 01140995201

العمر (تاريخ الميلاد) : 13-12-1976 المهنة : ربه منزل

الإقرار

أقر أنا الموقع أدناه أنني لا اتمتع بأي نظام علاج مثل :- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات أو أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي (.....) التوقيع: (.....)

المقرر بما فيه (.....) التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام مزيد من الأبحاث و الاشعات [7954] قسم العلاج : الاورام

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني الوظيفة : التوقيع :
الاسم : د/ندى حسن الوظيفة : التوقيع :
الاسم : د/نرمين رمضان الوظيفة : التوقيع :

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة رئيس اللجنة ومدير المستشفى.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

..... د/..... د/..... د/..... د/.....

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى : قسم :
جهة العلاج : نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: صفيه صبيحي مصطفى محمد National ID: 27612132101822	
--	---

تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
---	---

وبعد الكشف والفحوصات والمتابعة وجد أنه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

الجمهورية العربية السورية
الوكالة تحقيق الشخصية

صفحه
صبيحى مصطفى محمد
١٩ عبد المنعم رياض - جمال عبد الناصر
الجيزة - الجيزة

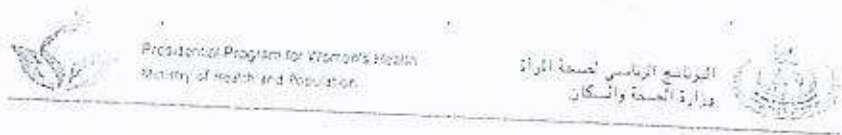
٢ ٧٦ ١٢ ١٣ ٢١ ٠ ١٨ ٢٢

KL6657867

٢٠٢٤/٠٣ ٢٧٦١٢١٣٢١٠١٨٢٢
رؤية منزل

أنش
مسلمة
متزوجة
حسام الدين طه عبد الحميد عامر
البطاقة سارية حتى ٢٠٣١/٠٣/٠٩

١



A-Patient ID:
1- Name : **صليحة صبحي مصطفى محمد** Sex: **Female** 3- Medical number: **83178411** Old Med : **289/2024**
2-ID number: **27612132101822** Patient Age : **47 Y**

Family history Family History +ve **Phone** 01140995261 **BMI** **REASON OF MDT**
marital status Marital Status MARRIED **NRS Score** **GROUP** **REASON OF MDT**
Co morbidity Co Morbidity free **Menopausal status** **Initial STAGE** **Date of MDT** 25/02/2024

Breast Imaging :
L.T BREAST MULTICENTERIC MALIGNANT LOOKING MASS WITH
IPSILATRAL FEW LEVEL 1 AND SINGLE LEVER II INDETERMINATE
AXILLARY LNS (BIRADS V) FOR HISTIOATHOLOGICAL CORRELATION
- UNREMARKABLE STUDY OF RIGHT BREAST (BIRADS I)

Clinical data
Mets Workup :
• Left breast malignant looking mass lesion and
parenchyma with pathological ipsilateral axillary
lymph nodes as described.
• Few bilateral scattered pulmonary nodules for follow up.
• No hepatic or osseous metastatic deposits.

BX	ER 8/8 BX COMMENT: IDC & FOCAL DCIS	PR 8/8 PR COMMENT:	Her 2 Neu 1 HER 2 Neu COMMENT:	SISH SISH COMMENT:	K167 15
3	N 1 M 0				

onammammography
L.T BREAST MASS 4 O'CLOCK
1.6X5X5.5 CM, OTHER MASS AT 10 OCLOCK 3.2 CM, INDETERMINATE AXILLARY LN
PATIENT HISTORY
T MRM:bifocal IDC GII
ocal DCIS component
ve LN 2/12

MDT OUTCOME & PLAN
OUTCOME & PLAN
ORT , for AC 4 cycles then TXT 4 cycles then AI +- Zoladex

EDICAL ONCOLOGY	SIGNATURE	RADIOLOGY
SURGERY	PATHOLOGY	NUTRITION
Psychiatric		

Name: صفيه صبحي مصطفى محمد			
MR. : 3797			
Detailed Medical Report			
		السيد	
تحيه طيبة وبعد,,			
		إيماء للطلب المقدم من	
بشأن تقرير طبي للمريض			
رقم طبي:		بقسم:	
نشرف بأن نوضح فيما يلي تقريراً عن حالته			
تاريخ وساعة الخروج :		تاريخ وساعة الدخول :	
وبعد الكشف والفحوصات والمتابعة وجد انه The patient was initially staged in 2024 as UPFRONT LT MRM T3 N 2/12 M0 and was planned for adjuvant therapy consisting of AC*4, TXT*4, PORTH, and AI. Follow-up imaging scheduled for October 2025 includes a brain MRI, which demonstrated ethmoidal sinusitis, and a bone scan that was reported as free of disease. On a subsequent visit, the patient was prescribed Zoladex and Arimidex to be continued for three months, with the medications dispensed by the pharmacy. At the most recent encounter, the patient again received Zoladex and Arimidex, and supportive treatment was ordered for three months. A bone scan performed in April 2026 was reported as free of disease. The current plan is to continue the hormonal therapy as previously outlined.			
تحريرا في ,			
التاريخ :		19-09-2026	
الوقت :		نهى سعيد علي محمد	
		الطبيب المعالج :	
يعتمد ,			



Patient Name	: الأستاذ / صفيه صبحي مصطفى محمد	Request No.	: 13004021
Patient No.	: 27612132101822	Request Date	: 25/12/2023
Path No.	: 4546/23	Verification Date	: 28/12/2023
Sex	: Female	Financial Category	: أورام دار السلام - نفقة الدولة
Age	: 47 Y	Department	: Out Patient
Referring Doc.	:	IHC No.	:
		Registered by	: marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Left breast mass

Nature of Specimen: Tru-cut biopsy

Technique:

Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:

ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Results:

IPT (Positive control showed appropriate reactivity):

ER: Strong nuclear reaction in 80% of tumor cells (Allred score 8/8).

PR: Strong nuclear reaction in 80% of tumor cells (Allred score 8/8).

Her2: Faint incomplete membrane reaction in >10% of tumor cells (score 1+).

Ki67: 15% nuclear reaction of tumor cells.

Conclusion:

Left breast mass, Tru-cut biopsy:

Invasive duct carcinoma, Grade II with focal DCIS.

PT:

ER: Positive (score 8/8).

PR: Positive (score 8/8).

Her2: Score 1+.

Ki67: Positive in 15% of tumor cells.

Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations, 2010 Allred score: proportion of cells staining (0,

Verified by :

Dr.Marwa mahmoud

Printing Date : 28/12/2023 12:58:58

Page 1 of 2

Printed By : Ali mostafa



Patient Name	الأستاذة / صفية صبحي مصطفى محمد	Request No.	13004021
Patient No.	27612132101822	Request Date	25/12/2023
Path No.	4546/23	Verification Date	28/12/2023
Sex	Female	Financial Category	اورام دار السلام - نفقة الدولة
Age	47 Y	Department	Out Patient
Referring Doc.			
		Registered by	.marwa mohamed youss

IMMUNOHISTOCHEMISTRY REPORT

no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature

Prof. Dr. Gamal Abdelaty

Dr. Toka Medhat

Verified by :

Dr.Marwa mahmoud

Printing Date : 28/12/2023 12:58:58

Page 2 of 2

Printed By : Ali mostafa



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt



Patient Name: Safeya Sobhy Moustafa	Date : 30/12/2023
Exam : C.T CHEST WITH CONTRAST	Age : 47 Year(s)
Medical ID: 4744/2023	Pat. ID: 45646

**POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST,
ABDOMEN & PELVIS REVEALED: -**

1. Chest:

- Left breast pathological enhancing parenchyma and mass measuring 6 x 2.9 cm and 2.2 x 2.3 cm respectively.
- right axillary level I and II pathologically enlarged lymph nodes the largest measures 2 x 1.9 cm.
- Prominent subcentimetric bilateral supra-clavicular lymph nodes.
- Few bilateral scattered tiny pulmonary nodules, the largest is noted at posterior segment of left upper lung lobe measuring 6 mm.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

2. Abdomen and pelvis:

- The liver is average in size with homogenous CT texture. No definite focal lesions or contour irregularities.
- Patent homogenously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.



Patient Name: Safeya Sobhy Moustafa	Date : 30/12/2023
Exam : C.T CHEST WITH CONTRAST	Age : 47 Year(s)
Medical ID: 4744/2023	Pat. ID: 45646

- No abdominal or pelvic masses or collections.
- 3. The scanned skeleton:
 - The scanned skeleton shows no definite worrisome lytic or sclerotic lesions.
 - No vertebral structural collapse or extra-osseous soft tissue component.

OPINION:

- *Left breast malignant looking mass lesion and parenchyma with pathological ipsilateral axillary lymph nodes as described.*
- *Few bilateral scattered pulmonary nodules for follow up.*
- *No hepatic or osseous metastatic deposits.*

Dr. Wafaa Ali, MSc.

Signature Prof. Menna Talla Hanafy

01/01/2024 05:59



Patient Name	: الأستاذة / صفية صبحي مصطفى محمد	Request No.	: 13000953
Patient No.	: 27612132101822	Request Date	: 21/01/2024
Path No.	: 289/24	Verification Date	: 29/01/2024
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 47 Y	Department	: Out Patient
Referreing Doc :		Registered by	: marwa mohamed youss

Pathology Report

Clinical Data: Left breast cancer, Upfront surgery.

Nature of the specimen: Left MRM specimen.

Gross Examination:

- Procedure: Left MRM specimen; measured 25x19x5 cm, skin covering measured 22x7 cm, nipple and areola were unremarkable grossly.
- Localization: Midline (6 oclock), and UIQ masses.
- Tumor extent
 - o Number: Two ill defined grayish white firm masses, 4.5 cm distance inbetween.
 - o Size: 3.5x3x2 cm and 1.8x1.8x1.5 cm, respectively.
 - o Skin involvement: 2 cm and 0.4 cm away from skin respectively.
- Distance from margins: 2 cm and 1 cm from deep margin respectively.
- Lymph nodes number and size: fibrofatty tissue measured 10x7 cm, enclosing 12 nodules measured up to 2.5 cm.
- Additional pathologic findings: N/A
- Immunohistochemistry technique: N/A
- Frozen section: N/A

Microscopic examination:

- Histological type: Bifocal Invasive duct carcinoma
- Grade (differentiation+ nuclear pleomorphism + mitosis): II (3+2+1)
- Angioinvasion: Positive
- Ductal carcinoma in situ: Focal (20%), (comedo and non comedo)
- Lymph node involvement: Positive meatstatic deposits in 2/12 axillary lymph nodes with no extra capsular extension.
- Resection Margins: Free
- Histologic response to therapy: N/A

Verified by :

walaa ghoniem





Patient Name	: الأستاذة / صفية صبحي مصطفى محمد	Request No.	: 13000953
Patient No.	: 27612132101822	Request Date	: 21/01/2024
Path No.	: 289/24	Verification Date	: 29/01/2024
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 47 Y	Department	: Out Patient
Referring Doc :		Registered by	: marwa mohamed youss

Pathology Report

- Additional pathologic findings: Free nipple, no perineural invasion.

Diagnosis:

Left breast cancer; Left MRM specimen:

- Bifocal Invasive duct carcinoma, grade II
- Focal DCIS component
- Free Nipple
- Free margins
- Positive metastatic deposits in 2/12 axillary lymph nodes (2/12)
- pT2 pN1.

Prof.Dr. Lobna Omar
Dr. Heba Hassan

Verified by :
walaa ghoniem





Patient Name: Safeya Sobhy Moustafa	Date : 08/01/2024
Exam : DUPPLEX VENS LEFT LEG	Age : 47 Year(s)
Medical ID: 4774/2023	Pat. ID: 45646

COLOR DOPPLER STUDY OF BOTH LOWER LIMBS VENOUS SYSTEM REVEALED: -

I- DEEP SYSTEM: -

- Patent common femoral, superficial femoral, popliteal and infrapopliteal veins showing normal compressibility and augmentation with saturated color flow Doppler.
- No evidence of DVT or venous obstruction.

II- SUPERFICIAL SYSTEM:

RIGHT AXIS:

- Incompetent sapheno-femoral junction showing mild reflux
- The long saphenous vein is seen dilated more evident at the lower thigh and opposite knee joint and connected with multiple dilated tortuous varicosities at the leg
- incompetent perforator of about 4mm thickness about 15cm above the MM
- Competent sapheno-popliteal junction.
- The short saphenous vein is seen patent with no evidence of varicosities.

LEFT AXIS:

- Incompetent sapheno-femoral junction showing marked reflux
- The long saphenous vein is seen dilated more evident at the lower thigh and opposite knee joint and connected with multiple dilated tortuous varicosities at the leg
- incompetent perforator of about 3mm&4mm thickness about 10cm & 15cm above the MM respectively
- Competent sapheno-popliteal junction.
- The short saphenous vein is seen patent with no evidence of varicosities.

N.B: Multiple subcutaneous varicosities are seen at both lateral side of the thighs and legs with no sizable connection to the superficial or deep venous systems on both sides

CONCLUSION:

- No DVT or superficial thrombosis at time of scan .
- Bilateral incompetent SFJ more at the left side.
- Bilateral lower limb primary axial varicose veins more evident on the left side as described.
- Bilateral mild extra axial varicosities

Signature Dr . Nermien Ramadan

PM 3:21 2024/01/08

SOBHY, S Patient ID: 123027078 DOB: 13-Dec-1976 Date: 29-Jul-2024

Patient Lastname: Sobhy

Patient Firstname: S

HIS Patient ID: 123027078

RIS Patient ID: 345047

Date of Birth: 13/12/1976

Procedure: 70030, Bone Scan

Medical Question:

Clinical Comment:

===== REPORT TEXT =====

99mTc MDP Bone Scan Report (29/07/2024)

* Whole Body Anterior and Posterior Planar Views Acquired 2Hrs. Post I.V Injection of 20mCi of 99mTc MDP
Revealed:

" No definite evidence of active osseous deposits could be seen over the surveyed skeleton.

" Degenerative changes are noted over the dorso-lumbar vertebrae.

" Arthritic changes are noted over both shoulder and both knee joints.

" Both kidneys are visualized.

Conclusion:

" No definite evidence of active metastatic osseous deposits could be seen all over the surveyed skeleton.

Much Obligated

Omnia Talaat,MD

===== REPORT TEXT ENDE =====



Patient Name	: الأستاذ / صفية صبحي مصطفى محمد	Request No.	: 13004021
Patient No.	: 27612132101822	Request Date	: 25/12/2023
Path No.	: 4546/23	Verification Date	: 28/12/2023
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 47 Y	Department	: Out Patient
Referring Doc :		Registered by	: marwa mohamed youss

Pathology Report

Clinical Data: Left breast mass.

Nature of Specimen: Tru-cut biopsy, collected at 24/12/2023, 12.10 pm.

Gross Examination:

- Multiple grayish white tissue cores up to 1 cm, totally submitted.

Microscopic Examination:

- * Histological type: Invasive duct carcinoma
- * Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+1) Grade II.
- * Ductal carcinoma in situ: Focal
- * Perineural invasion: No
- * Angioinvasion: No

Diagnosis:

Left breast mass, Tru-cut biopsy:

Invasive duct carcinoma, Grade II, with focal DCIS.

For further IHC (ER, PR, Her2, Ki67).

Signature

Prof. Dr. Gamal Abdelaty
Dr. Toka Medhat

Verified by :

Dr.Marwa mahmoud

Printing Date : 28/12/2023 12:59:03

Page 1 of 1

Printing By : Ali mostafa



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt



Patient Name: Safeya Sobhy MOSTAFA	Date : 14/12/2024
Exam : Item 1	Age : 40 Year(s)
Medical ID: MOSTAFA^L.S.S	Pat. ID: QC_45646

MRI EXAMINATION OF THE DORSAL AND LUMBO-SACRAL SPINE

Technique of examination:

- Sagittal T1 & T2 WIs
- Axial T2 WIs
- No IV contrast was given.

MRI Findings:

- Straightened lumbar lordotic curve.
- Preserved sagittal diameter of the bony dorso-lumbar spinal canal.
- Mild spondylodegenerative changes of the dorso-lumbar spine manifested by:
 - ❖ Anterior marginal osteophytic lipping of the vertebral end plates
 - ❖ Few small Schmorl's nodes of the opposing vertebral end plates
 - ❖ Diminished height and bright T2 signal intensity of the discs denoting their degeneration.
- L5-S1 and to less extent L4-5 posterior disc bulges are seen effacing the epidural fat, flattening the theca and mildly encroaching both exit neural foramina
- No definite marrow infiltrative lesions.
- Normal MRI appearance of the conus medullaris and cauda equine nerve roots.
- No pre-vertebral or para-vertebral signal abnormalities.

Opinion:

- ***Mild spondylodegenerative changes of the dorso-lumbar spine with L4-5 and L5-S1 posterior disc bulges, their significance should be clinically correlated.***

Dr. Alaa R. Hani, M.Sc., FEBR
Prof. Menna Tallah Hanafy, M.D.



Patient Name: Safeya Sobhy Moustafa	Date : 06/03/2025
Exam : ultrasound BREAST (BOTH SIDE)	Age : 48 Year(s)
Medical ID: 4774/2023	Pat. ID: 45646

RIGHT BREAST MAMMOGRAPHY WITH BILATERAL US

Indication:

Patient had left MRM

Technique

- right breast MLO and CC views
- Bilateral ultrasound

Report:

- scattered fibroglandular densities of the right breast (ACR B)
- No spiculated masses or grouped microcalcifications.
- Complementary ultrasound also showed no right breast solid or cystic masses
- Normal skin thickness and nipple-areola complex, with average caliber of the retro-areolar ducts
- Nonspecific right axillary LN, with preserved fatty hila and uniform cortical thickness.
- Free left operative bed with no evidence of recurrent mass lesions
- Free left infra and supra clavicular regions.

Opinion:

- Left MRM with free operative bed with no evidence of recurrent mass lesions
- No right breast suspicious findings (BIRADS 2)

Yours truly

Signature Dr. Engy Abdallah

06/03/2025 2:08 PM